

DEXMIN SYRUP 2mg/5ml

Composition:

Each 5 ml contains

Dexchlorpheniramine maleate 2mg

Sodium benzoate 5mg (preservative)

Description:

A red colored syrup having fruity flavor

Pharmacodynamics:

Dexchlorpheniramine is a propylamine derived antihistamine. Its use in the treatment of allergy is due to its action of competing with histamine H₁-receptor site on effector cells. Antihistamines antagonize, in varying degrees, most of the pharmacological effects of histamine, including urticaria, pruritus and vasodilation. Dexchlorpheniramine also prevents responses to acetylcholine that are mediated via muscarinic receptors resulting in drying of the nasal mucosa.

Pharmacokinetics:

Dexchlorpheniramine is absorbed relatively slowly from the gastrointestinal tract, reaching peak plasma concentrations about 2-6 hour after administration. The peak effect occurs at 6 hours after administration. The onset of action is between 15-60 minutes. Bioavailability is between 25-50%. About 72% of dexchlorpheniramine in the circulation is bound to plasma.

Unchanged drug and metabolites are excreted primarily in urine but trace amounts were also found in the faeces.

Indication:

Symptomatic treatment of perennial and seasonal allergic rhinitis, vasomotor rhinitis, allergic conjunctivitis, mild, uncomplicated allergic skin manifestations of urticaria and angioedema, amelioration of allergic reactions to blood or plasma.

Therapy for anaphylactic reactions adjunctive to epinephrine and other standard measures after the acute manifestations have been controlled. Skin conditions eg, allergic eczema, atopic dermatitis, contact dermatitis, insect bites, dermographism and drug reactions are often relieved by Dexmin.

Recommended dose:

Adults and children ≥ 12 years: 5 ml, 3-4 times daily

Children 6-12 years: 2.5ml, 3-4 times daily

Children 2-6 years: 1.25ml, 3-4 times daily

Use is not recommended in premature and full-term neonates.

Contraindication:

Dexchlorpheniramine should not be given to premature infants or neonates, patients receiving MAO inhibitor therapy and to patients who have shown a previous hypersensitivity.

Warning and precautions:

Dexchlorpheniramine may cause drowsiness, dizziness, blurred vision and psychomotor impairment in some patients. This will affect ability to drive or operate machinery and precipitate or aggravate urinary retention in patients with bladder neck obstruction, symptomatic prostatic hypertrophy and predisposition to urinary retention. Dexchlorpheniramine should be used with care in patients with pyloroduodenal obstruction, severe cardiovascular disorders and epilepsy. Increased intraocular pressure may precipitate an attack of angle-closure glaucoma.

Effects on the Ability to Drive or Operate Machinery: Patients should be warned about engaging in activities requiring mental alertness eg, driving a car or operating appliances, machinery, etc.

Warning: Not suitable to be used in children below 2 years old.

For children between 2 to 6 years old – use with caution and follow the advice given by the doctor or pharmacist.

Interaction with other medicaments:

Monoamine oxidase (MAO) inhibitors prolong and intensify the effects of antihistamines; severe hypotension may occur. Concomitant use of antihistamines with alcohol, tricyclic antidepressants, barbiturates or other central nervous system depressants may potentiate the sedative effect of dexchlorpheniramine. The action of oral anticoagulants may be inhibited by antihistamines.

Pregnancy and lactation:

Safety during pregnancy has not been established.

It is not known whether Dexchlorpheniramine maleate is excreted in human milk, and therefore caution should be exercised when administered to nursing mothers.

Side effects:

The physician should be alerted to the possibility of any adverse effects associated with antihistaminic drugs. Slight to moderate drowsiness is the most frequent adverse effect of dexchlorpheniramine maleate. Other possible adverse effects of antihistamines include cardiovascular, hematologic, neurologic, gastrointestinal, genitourinary and respiratory reactions. General adverse effects eg, urticaria, drug rash, anaphylactic shock, photosensitivity, excessive perspiration, chills, dryness of mouth, nose and throat have been reported.

Overdosage:

In humans, the estimated lethal dose of dexchlorpheniramine is 2.5-5 mg/kg.

Symptoms: Antihistamine overdosage effects may vary from central nervous system depression (sedation, apnea, diminished mental alertness, cardiovascular collapse) to stimulation (insomnia, hallucinations, tremors or convulsions) to death. Other signs and symptoms may be dizziness, tinnitus, ataxia, blurred vision and hypotension. Stimulation is particularly likely in children, as are atropine-like signs and symptoms (dry mouth; fixed, dilated pupils; flushing, hyperthermia; and gastrointestinal symptoms).

Treatment: In the event of overdosage, emergency measures should be performed. The patient should be induced to vomit even if emesis has occurred spontaneously. Pharmacologically induced vomiting by the administration of ipecac syrup is a preferred method. However, vomiting should not be induced in patients with impaired consciousness. The action of ipecac is facilitated by physical activity and by the administration of 240-360 mL of water. If emesis does not occur within 15 min, the dose of ipecac should be repeated. Precautions against aspiration must be taken, especially in infants and children. Following emesis, any drug remaining in the stomach may be absorbed by activated charcoal administered as a slurry with water. If vomiting is unsuccessful or a contraindication, gastric lavage should be performed. Isotonic and ½ isotonic saline are the lavage solutions of choice.

Saline cathartics draw water into the bowel by osmosis and therefore may be valuable for their action in rapid dilution of bowel content. Dialysis is of little value in antihistamine poisoning.

After emergency treatment, the patient should continue to be medically monitored.

Treatment of the signs and symptoms of overdosage is symptomatic and supportive. Stimulants (analeptic agents) should not be used.

Vasopressors may be used to treat hypotension. Short-acting barbiturates, diazepam or paraldehyde may be administered to control seizures. Hyperpyrexia, especially in children, may require treatment with tepid water sponge baths or a hypothermic blanket. Apnea is treated with ventilatory support.

Storage:

Store in a cool dry place below 30°C.

Keep container tightly closed.

Shelf life:

3 years

Presentation:

HDPE bottles: 60ml, 90ml, 120ml

Manufactured by:

Noripharma Sdn. Bhd.

Lot 5030, Jalan Teratai,

5 ½ Mile off Jalan Meru,

41050 Klang, Selangor